SACTCHECK · NCCP 00525

My complete treatment handbook

Ribociclib + endocrine therapy · metastatic

All information can be read on paper. Keep this with your treatment record.

> Use the urgent instructions and contact numbers in the following companion pages. Example telephone numbers must be replaced by your team before patient use. Your individual treatment plan takes precedence.

Contents

1. Your three-page treatment companion — page 2
2. Full treatment reference: medicines, schedule, tests, symptoms and follow-up — page 5
3. Detailed treatment and monitoring plan — page 19
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6. Your treatment passport and symptom record — page 25
7. Wallet card and phone instructions — page 30

SACTCheck v0.84.33 · Clinical, pharmacy and patient review pending. Formal consent is supplied separately.

Ribociclib + endocrine therapy · metastatic

Ribociclib with endocrine treatment to control advanced breast cancer.

HR-positive / HER2-negative metastatic breast cancer · NCCP 00525 v6 · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

My overall treatment

Ribociclib daily on Days 1–21, then seven days off. Endocrine treatment follows its own schedule. Continue while beneficial and tolerated.

Days 1–21
Take prescribed ribociclib; continue endocrine treatment.

Days 22–28
No planned ribociclib; endocrine treatment continues.

A typical 28-day cycle

Day 1–28
Follow the phase and prescribed schedule

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Bloods, electrolytes and ECG** Required by regimen	Before starting
Cycles 1–6	**Blood count, kidneys and liver** Required by regimen	Every two weeks in cycles 1–2; before cycles 3–6
During	**Electrolytes and ECG** Required by regimen	Electrolytes before cycles 1–6; ECG at cycle 1 Day 14
Later / review	**Bloods, ECG and response assessment** Team-agreed plan	Further tests as indicated; scans follow your plan

Between treatments

- Report fever, fainting, palpitations or new breathing symptoms.

Confirm the endocrine partner and injection appointments; do not replace missed doses.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Heart rhythm / QT

Ribociclib

Baseline ECG and an ECG around Cycle 1 Day 14. Fainting or palpitations need urgent advice.

3 Low neutrophils / infection

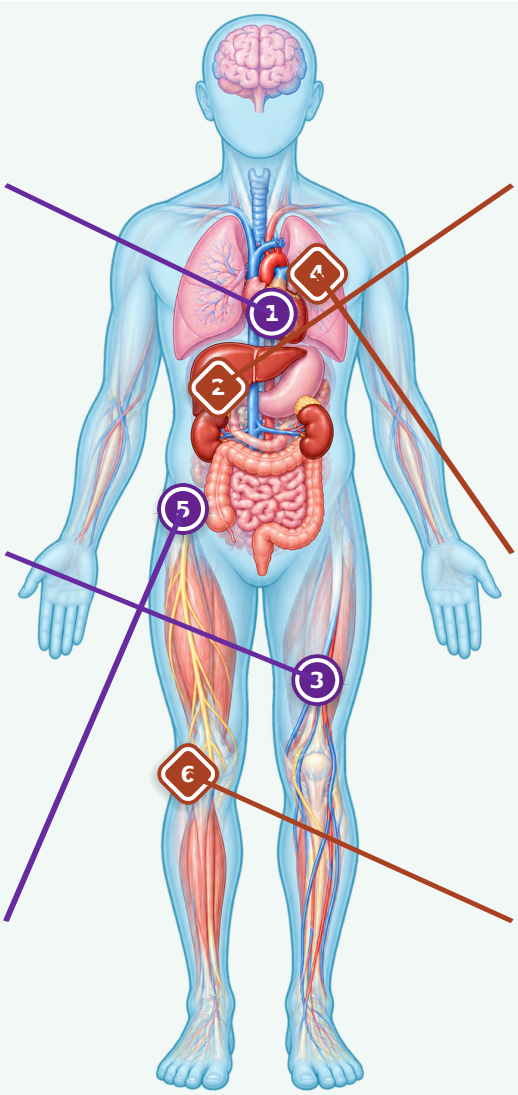
Ribociclib

Blood counts guide whether the next cycle can start. Fever, shivers or sudden illness: contact oncology immediately.

5 Severe rash or blistering

Ribociclib

A widespread painful rash, blisters or peeling skin needs urgent assessment.



2 Liver injury

Ribociclib

Keep liver blood tests. Yellow skin / eyes, dark urine or marked upper-abdominal discomfort: call promptly.

4 Lung inflammation

Ribociclib · uncommon but important

New cough or breathlessness needs prompt assessment, even between visits.

6 Joints and bone health

Aromatase inhibitor / ovarian suppression

Discuss joint symptoms and keep the agreed bone-risk and DXA plan.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathlessness, chest symptoms, severe diarrhoea, inability to drink, jaundice, a painful swollen leg or a severe rash promptly. You do not need to identify which medicine caused the symptom.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Keep your treatment details to hand

HR-positive / HER2-negative metastatic breast cancer · NCCP 00525 v6 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Ribociclib + endocrine therapy

Ribociclib Days 1–21 of 28; endocrine partner has a separate continuing schedule.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

NCCP 00525 · VERSION 6

Ribociclib + endocrine therapy · metastatic

HR-positive / HER2-negative metastatic breast cancer · NCCP 00525 v6 · Document revised 4 November 2026

> **Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated.

Oral treatment with a planned rest week

How this medicine supports your treatment

Ribociclib with an aromatase inhibitor or fulvestrant aims to control advanced HR-positive/HER2-negative breast cancer. The exact endocrine partner depends on previous treatment and menopausal status.

CDK4/6 INHIBITOR

Ribociclib

Slows the cell-division checkpoint

CDK4/6 checkpoint → Checkpoint inhibited → Cell cycling slows

Ribociclib blocks CDK4 and CDK6, proteins that help cells move through the cycle of division. Endocrine therapy reduces the hormone signal that can drive HR-positive breast cancer.

Keep blood and ECG appointments. Fever, jaundice, fainting, palpitations, severe rash or new breathlessness need prompt advice.

ENDOCRINE PARTNER

AI or fulvestrant

Reduces oestrogen-driven growth

Oestrogen pathway → AI or receptor blockade → Growth signal reduced

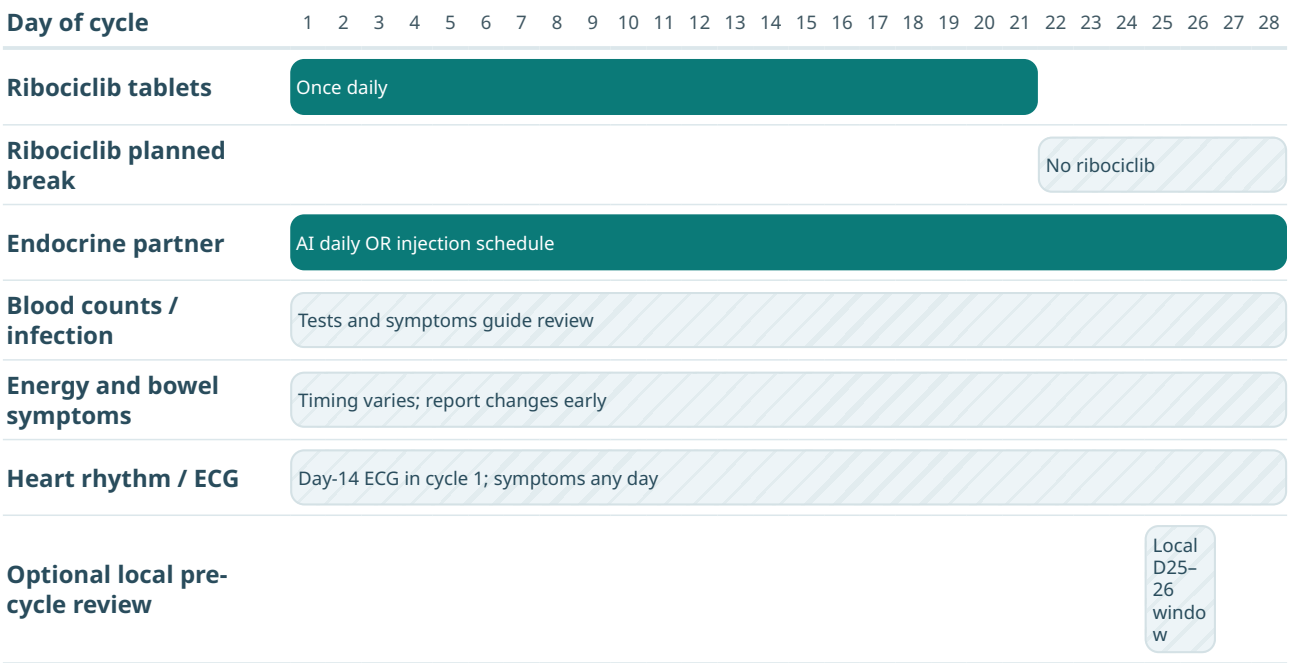
An aromatase inhibitor (AI) lowers oestrogen production; fulvestrant blocks and reduces oestrogen receptors. These are alternative endocrine partners, selected by the team. Ovarian suppression is needed in pre/perimenopausal patients.

The endocrine medicine continues during the ribociclib week off. Report troublesome joint, menopausal or injection-site symptoms.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated.



Treatment days: Ribociclib is taken once daily on Days 1–21, followed by seven days off, in a 28-day cycle. The endocrine partner continues according to its own prescription. Treatment continues while it is helping and tolerable.

Read the phase labels: The prescription and team's instructions determine your actual doses and any interruptions.

The bigger picture: Use the whole-treatment journey for surgery, radiation, scans and longer-term checks.

Local pre-cycle review: Day 25–26 can be a local blood / review window. It does not replace the required early checks or authorise an automatic restart.

> **Why these bands stay broad:** Treatment bands show the source schedule. Symptom bands show possible patterns, not measured probabilities; serious effects can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00525 v6 · official regimen ↗️

> **There are no guaranteed "safe days".** Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override the timeline or diary.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Columns show sequence and general intervals, not equal periods or booked appointments. The team confirms who arranges each test, who reviews results and who authorises treatment or restart.

Check	Before first dose	Cycles 1–2	Cycles 3–6	Later treatment	Completion / change
Ribociclib and endocrine partner	Confirm metastatic 600 mg / AI or fulvestrant plan	Days 1–21 on; Days 22–28 off; fulvestrant loading visits if chosen	Repeat if cleared; endocrine partner continues	While beneficial and tolerated	Review endocrine continuation separately
Blood counts / renal / liver	Baseline profiles	Every two weeks in both cycles	Before each cycle	As clinically indicated; more often if abnormal	Follow abnormalities before restart or after stopping
ECG / heart rhythm	Baseline ECG; check QTcF eligibility	Repeat around cycle 1 Day 14	Further ECG when clinically indicated	Repeat for symptoms, QT change or interacting medicines	No automatic restart after a QT-related hold
Electrolytes	Potassium, calcium, magnesium and phosphorus	Before each cycle; correct abnormalities	Before each cycle through cycle 6	As clinically indicated	Review unresolved abnormalities
Bone / endocrine health	Assess AI/OFS bone and menopausal risks	Support symptoms and adherence	AI/OFS and any injections follow their own plan	Repeat bone assessment according to risk	Longer endocrine and bone plan continues
Cancer response	Baseline disease assessment	Assess new symptoms promptly	Imaging according to consultant plan	Continue response and symptom review	Progression prompts new treatment decision

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

THE WHOLE TREATMENT JOURNEY

Ribociclib + endocrine therapy · metastatic: treatment and checks

HR-positive / HER2-negative metastatic breast cancer

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before first dose	Cycles 1–2	Cycles 3–6	Later treatment	Completion / change
Ribociclib and endocrine partner NCCP 00525 · SMPC	Confirm metastatic 600 mg / AI or fulvestrant plan	Days 1–21 on; Days 22–28 off; fulvestrant loading visits if chosen	Repeat if cleared; endocrine partner continues	While beneficial and tolerated	Review endocrine continuation separately
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Cancer response NCCP 00525 · SMPC	Baseline disease assessment	Assess new symptoms promptly	Imaging according to consultant plan	Continue response and symptom review	Progression prompts new treatment decision

What each check is for

1

Before the first dose

Before first dose

Check eligibility, organ function and the exact treatment/support plan.

Ask / act: Confirm who arranges each check and who reviews it.

Sources: NCCP 00525 · SMPC

2

Early toxicity checks

Cycles 1–2

Important toxicity can begin before the first routine review.

Ask / act: Keep early appointments and use the agreed symptom-contact plan.

Sources: NCCP 00525 · SMPC

3

Benefit, tolerability and adherence

Later treatment

Longer-term treatment still needs clinical review.

Ask / act: Ask about symptom support, results and the next decision.

Sources: NCCP 00525 · SMPC

4

Completion or treatment change

Completion / change

The targeted medicine and endocrine partner may have different stop dates.

Ask / act: Confirm each medicine and the ongoing follow-up plan.

Sources: NCCP 00525

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21

Your treatment aim

Ribociclib with an aromatase inhibitor or fulvestrant aims to control advanced HR-positive/HER2-negative breast cancer. The exact endocrine partner depends on previous treatment and menopausal status.

Who this treatment is for

HR-positive/HER2-negative locally advanced or metastatic disease with an AI or fulvestrant in the exact NCCP indication. NCCP page 1 and page 2 do not fully align on menopause and previous advanced-disease treatment; preserve consultant reconciliation.

NCCP 00525 v6 · official regimen

Schedule and duration

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated. There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision.

NCCP 00525 v6 · official regimen

Checks before and during treatment

Baseline tests and ongoing tests have different purposes.

Before the first dose

FBC, renal and liver profile, potassium, calcium, magnesium and phosphorus. ECG before initiation; QTcF must be below 450 ms. Correct electrolyte abnormalities and review QT/CYP3A interactions. Confirm endocrine partner, menopause/ovarian suppression, prior treatment and the NCCP indication/eligibility discrepancy.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

During treatment

FBC, renal and liver profile every two weeks for cycles 1–2, then before cycles 3–6, then as clinically indicated. Electrolytes before each of the first six cycles, then as clinically indicated; correct abnormalities. Repeat ECG around Day 14 of cycle 1, then as clinically indicated. Increase liver monitoring for grade 2 or worse abnormalities. Review fever, jaundice, palpitations/fainting, pulmonary and severe skin symptoms; response imaging follows the advanced-disease plan.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Living with treatment

Supportive medicines and the endocrine partner have their own plan.

Nausea support

NCCP classifies ribociclib as oral moderate–high emetic risk. The team chooses preventive or rescue antiemetics using local policy, symptoms, QT risk and drug interactions; do not reuse another regimen's prescription.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Electrolytes and heart rhythm

Correct abnormal potassium, calcium, magnesium and phosphate as directed. Attend the baseline and cycle 1 Day-14 ECG; fainting or palpitations need urgent advice.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Counts and liver tests

Keep the early two-weekly blood tests and subsequent monitoring. Report fever, shivers, jaundice or dark urine promptly.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Endocrine treatment

The AI or fulvestrant, and ovarian suppression when prescribed, continue on their own timetable. Discuss joint, menopausal, bone and injection-site symptoms.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Daily administration

Avoid grapefruit and check interactions. Do not replace a missed or vomited dose. A clinical hold requires explicit instructions about which medicine stops and who authorises restart.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

What the studies tell us

Studies of ribociclib with hormone therapy in defined advanced-breast-cancer groups showed a survival benefit. The studies used different hormone partners and included different menopausal and prior-treatment groups. Ask which study matches your plan; the results do not set an individual time limit.

How to interpret this

Study results describe groups with defined treatment and eligibility. Ask about your own likely benefits, important harms, alternatives and remaining uncertainty.

MONALEESA-2 final OS, Hortobagyi et al 2022 · MONALEESA-2 updated PFS, Hortobagyi et al 2018 · MONALEESA-3 updated overall-population OS, Slamon et al 2021 · MONALEESA-7 exploratory extended OS, Lu et al 2022

When the plan changes

There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision.

Continue, hold or stop

The team confirms the plan for each medicine and the next review. Do not restart a clinically held treatment from this educational timeline.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Practical support

Nausea support

NCCP classifies ribociclib as oral moderate–high emetic risk. The team chooses preventive or rescue antiemetics using local policy, symptoms, QT risk and drug interactions; do not reuse another regimen's prescription.

Electrolytes and heart rhythm

Correct abnormal potassium, calcium, magnesium and phosphate as directed. Attend the baseline and cycle 1 Day-14 ECG; fainting or palpitations need urgent advice.

Counts and liver tests

Keep the early two-weekly blood tests and subsequent monitoring. Report fever, shivers, jaundice or dark urine promptly.

Endocrine treatment

The AI or fulvestrant, and ovarian suppression when prescribed, continue on their own timetable. Discuss joint, menopausal, bone and injection-site symptoms.

Daily administration

Avoid grapefruit and check interactions. Do not replace a missed or vomited dose. A clinical hold requires explicit instructions about which medicine stops and who authorises restart.

Taking tablets / attending treatment

MONALEESA-2, MONALEESA-3 and MONALEESA-7: separate populations and partners

Studies describe groups. Ask how closely the treatment and study population match your situation.

Your team can discuss the strength and limitations of the evidence alongside treatment risks and alternatives.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report new breathlessness, chest symptoms, severe diarrhoea, inability to drink, jaundice, a painful swollen leg or a severe rash promptly. You do not need to identify which medicine caused the symptom.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

Questions to ask

How long does treatment last?

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated. There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision.

Which checks happen before and during treatment?

FBC, renal and liver profile, potassium, calcium, magnesium and phosphorus. ECG before initiation; QTcF must be below 450 ms. Correct electrolyte abnormalities and review QT/CYP3A interactions. Confirm endocrine partner, menopause/ovarian suppression, prior treatment and the NCCP indication/eligibility discrepancy. FBC, renal and liver profile every two weeks for cycles 1–2, then before cycles 3–6, then as clinically indicated. Electrolytes before each of the first six cycles, then as clinically indicated; correct abnormalities. Repeat ECG around Day 14 of cycle 1, then as clinically indicated. Increase liver monitoring for grade 2 or worse abnormalities. Review fever, jaundice, palpitations/fainting, pulmonary and severe skin symptoms; response imaging follows the advanced-disease plan.

Do I need to enter dates?

No. This map shows general timing without personal information. Your hospital arranges actual appointments; ask who books each test and who reviews its result.

What if a dose is missed or treatment is held?

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription. A hold may affect one medicine rather than the whole combination; confirm each component.

What does the evidence mean for me?

Studies of ribociclib with hormone therapy in defined advanced-breast-cancer groups showed a survival benefit. The studies used different hormone partners and included different menopausal and prior-treatment groups. Ask which study matches your plan; the results do not set an individual time limit.

What happens after treatment changes or ends?

There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision. Keep the separately agreed disease follow-up, endocrine and bone-health plan.

Explain the plan back: Which medicine is taken when, which test is due next, who reviews it, and which symptoms mean you should call today?

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00525 v6 · official regimen
- NICE NG101 · bone health and follow-up
- HSE · chemotherapy side effects and urgent advice
- ASCO · biomarkers in metastatic breast cancer
- ESMO metastatic breast cancer guideline · response assessment
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · PICC patient information
- eviQ · implanted port patient information
- Novartis · Kisqali product information, updated September 2026
- MONALEESA-2: ribociclib with letrozole · primary report
- EMA Kisqali current product information; checked 2026-10-25
- MONALEESA-2 final OS, Hortobagyi et al 2022
- MONALEESA-2 updated PFS, Hortobagyi et al 2018
- MONALEESA-3 updated overall-population OS, Slamon et al 2021
- MONALEESA-7 exploratory extended OS, Lu et al 2022

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

Written source addresses

https://healthservice.hse.ie/documents/6629/525_v6_Ribociclib_Metastatic_hghzyyD.pdf

https://www.ema.europa.eu/en/documents/product-information/kisqali-epar-product-information_en.pdf

https://www.nice.org.uk/guidance/ng101/chapter/Recommendations

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://ascopubs.org/doi/10.1200/JCO.2015.61.1459

https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext

https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk

https://www.eviq.org.au/p/543

https://www.eviq.org.au/p/507

https://www.medicines.org.uk/emc/product/8110/smpc

https://pubmed.ncbi.nlm.nih.gov/35263519/

https://pubmed.ncbi.nlm.nih.gov/29718092/

https://pubmed.ncbi.nlm.nih.gov/34102253/

https://pubmed.ncbi.nlm.nih.gov/34965945/

Ribociclib + endocrine therapy · metastatic

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BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

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Electrolytes	Potassium, calcium, magnesium and phosphorus	Before each cycle; correct abnormalities	Before each cycle through cycle 6	As clinically indicated	Review unresolved abnormalities
Bone / endocrine health	Assess AI/OFS bone and menopausal risks	Support symptoms and adherence	AI/OFS and any injections follow their own plan	Repeat bone assessment according to risk	Longer endocrine and bone plan continues
Cancer response	Baseline disease assessment	Assess new symptoms promptly	Imaging according to consultant plan	Continue response and symptom review	Progression prompts new treatment decision

THE WHOLE TREATMENT JOURNEY

Ribociclib + endocrine therapy · metastatic: treatment and checks

HR-positive / HER2-negative metastatic breast cancer

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Treatment, tests and reviews across the course

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What each check is for

1 Before the first dose

Before first dose

Sources: NCCP 00525 · SMPC

2 Early toxicity checks

Cycles 1–2

Sources: NCCP 00525 · SMPC

3 Benefit, tolerability and adherence

Later treatment

Sources: NCCP 00525 · SMPC

4 Completion or treatment change

Completion / change

Sources: NCCP 00525

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.21 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

NCCP 00525 · clinician draft

Ribociclib + endocrine therapy · metastatic

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Separate scripts for each phase

Choose the active phase in the clinician workflow. Each script has its own medicines, timing and review checks.

- **Ribociclib + endocrine therapy · metastatic · supportive-care plan** — Oral moderate–high Ribociclib + AI or fulvestrant

https://sactcheck.com/patient/00525/supportive-care.html

Ribociclib + endocrine therapy · metastatic

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated.

Make treatment fit into daily life

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription.

Before starting

Confirm the exact medicine, strength, endocrine partner, baseline tests and first review appointments. Ask who arranges tests and who reviews the results.

Taking treatment

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription.

Your endocrine partner has a separate timetable

An aromatase inhibitor is normally taken daily through the targeted-medicine rest week. If fulvestrant is the chosen partner, the usual schedule is 500 mg by intramuscular injection on Days 1, 15 and 29, then once monthly; confirm the prescribed dose and appointment dates. Any ovarian-suppression injections have their own schedule.

Treatment duration

There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision.

Supply and appointments

Request the next supply before tablets run out. Blood tests and any ECG or injections need their own appointments. This oral treatment does not itself require a central line or infusion pump.

If treatment is held

Ask which medicine is held, whether the endocrine partner continues, what needs to recover, when the next review is due and who will authorise restart.

Support between appointments

Keep the oncology contact details and prescribed symptom medicines accessible. New severe symptoms need prompt assessment rather than waiting for the next blood test.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00525 v6 · official regimen
- NICE NG101 · bone health and follow-up
- HSE · chemotherapy side effects and urgent advice
- ASCO · biomarkers in metastatic breast cancer
- ESMO metastatic breast cancer guideline · response assessment
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · PICC patient information
- eviQ · implanted port patient information
- Novartis · Kisqali product information, updated September 2026
- MONALEESA-2: ribociclib with letrozole · primary report
- EMA Kisqali current product information; checked 2026-10-25
- MONALEESA-2 final OS, Hortobagyi et al 2022

- MONALEESA-2 updated PFS, Hortobagyi et al 2018
- MONALEESA-3 updated overall-population OS, Slamon et al 2021
- MONALEESA-7 exploratory extended OS, Lu et al 2022

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

NCCP 00525 · VERSION 6

Ribociclib + endocrine therapy · metastatic

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Your treatment and review passport

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal CTCAE assessment. Urgent symptoms override the form.

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report new breathlessness, chest symptoms, severe diarrhoea, inability to drink, jaundice, a painful swollen leg or a severe rash promptly. You do not need to identify which medicine caused the symptom.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

Your complete treatment reference

https://sactcheck.com/patient/00525/

Public information only. No personal information is encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

Ribociclib + endocrine therapy · metastatic

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Nausea	☐	☐	☐	
Fatigue	☐	☐	☐	
Skin / nail changes	☐	☐	☐	
Muscle / joint pain	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

> Report new breathlessness, chest symptoms, severe diarrhoea, inability to drink, jaundice, a painful swollen leg or a severe rash promptly. You do not need to identify which medicine caused the symptom. Emergency symptoms: 112 / 999.

Ribociclib + endocrine therapy · metastatic

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Muscle / joint pain	☐	☐	☐	

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_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

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_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

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Keep your treatment details to hand

HR-positive / HER2-negative metastatic breast cancer · NCCP 00525 v6 · Document revised 4 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Ribociclib + endocrine therapy

Ribociclib Days 1–21 of 28; endocrine partner has a separate continuing schedule.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.32 · NCCP 00525 v6 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.